

ACUTE LIMB ISCHEMIA (ALI) — Revision Table

Bold = high-yield.

DEFINITION & ETIOLOGY	
Topic	Key points
Definition	Sudden ↓arterial perfusion; <2 weeks (>2wks = CLTI); 6-hour reversible window
Etiology	Embolism (cardiac — AF, mural thrombus) , acute-on-chronic thrombosis, graft thrombosis, trauma, aneurysm
Emboli sources	Cardiac: AF (LA appendage), post-MI mural thrombus , prosthetic valve, paradoxical (PFO)
Mortality	9–22%; limb salvage 70–90% if <6h ; thrombotic worse than embolic

CLINICAL & CLASSIFICATION	
Topic	Key points
6 Ps	Pain, Pallor, Pulselessness, Poikilothermia, Paraesthesia, Paralysis (late)
Sequence	Sensory → motor → muscle necrosis
Rutherford	I viable; IIA marginal (toes); IIB immediate threat (revascularize NOW); III irreversible (amputation)
Investigations	Mostly clinical; CK; DSA gold standard ; don't delay revascularization

MANAGEMENT	
Topic	Key points
Initial	Systemic heparin (UFH 100 IU/kg) , IV analgesia, hydration, monitor renal (myoglobinuria)
Endovascular	Thrombolysis (tPA) for I-IIA , mechanical thrombectomy
Open	Fogarty embolectomy (embolic, normal artery) ; bypass/endarterectomy (thrombotic)
Reperfusion complications	Compartment syndrome, rhabdomyolysis (hyperK, acidosis), myoglobinuria → AKI
Arterial trauma	Hard signs (1 = surgery): pulsatile bleed, absent pulse, expanding haematoma, bruit/thrill ; soft signs = investigate; ATLS; fasciotomy